FAX

Bayfront University Medical Center

1 University Health Plaza, Orange County, CA
Case Management Department

To	Illumination Health + Home, Recuperative Care Referrals
Fax	888-555-0155
From	Rachel Imani, RN Case Manager
Phone	714-555-0189
Date	08/27/2026 11:05
Re	Recuperative care referral

MESSAGE

Recuperative care referral. 66-year-old man with pneumonia and COPD. Off oxygen since 08/26, on oral antibiotics, walking on his own. No home oxygen needed. Unhoused. Ready 08/28.

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Recuperative Care Referral Form

Illumination Health + Home. Submit this form with supporting documents by fax to 888-555-0155.

REFERRING PARTY

Referring Hospital	Bayfront University Medical Center	**Dept/Floor**	5 Tower Pulmonary
SW/CM/RN	Rachel Imani	**Phone/Pager #**	714-555-0189
Email	rimani@bayfrontumc.example.org	**Nursing Station #**	714-555-0130
Authorized by	Samuel Oyelaran, MD	**Phone #**	714-555-0130

PATIENT INFORMATION

Patient Name	HAROLD BENNING	**MRN**	SYN-407192
Date of Birth	01/30/1960	**Ethnicity**	White
Insurance	Medi-Cal managed care plan	**Preferred Language**	English

Gender: ☒ Male ☐ Female ☐ Transgender ☐ Other Last 4 # of Social Security: xxx-xx-0000

Housing Situation: ☒ Currently Unhoused ☐ 4+ episodes of homelessness in the past 3 years

HOSPITAL ADMISSION DETAILS

Chief Complaint / Admitting Diagnosis: Community-acquired pneumonia, right lower lobe, with COPD exacerbation

Admit Date: 08/24/2026 Any Wounds? ☐ Yes ☒ No

If yes, explain the number, location, size and stage:

Independent with Wound Care? ☐ Yes ☐ No If no, will Home Health be ordered? ☐ Yes ☐ No

Was PPD/TB Test or Chest X-Ray Performed? ☒ Yes ☐ No Date: 08/24/2026 Results: CXR: right lower lobe pneumonia

HISTORY OF HEALTH & LIVING SITUATION

O2	☐ Yes	☒ No	Active Infection	☒ Yes	☐ No
Self Administer All Meds	☒ Yes	☐ No	Ambulatory	☒ Yes	☐ No
Continent of Bowel & Bladder	☒ Yes	☐ No	Assistive Device	☐ Yes	☒ No
Colostomy/Ileostomy	☐ Yes	☒ No	Hospice	☐ Yes	☒ No
Foley Catheter	☐ Yes	☒ No	Car	☐ Yes	☒ No
Diabetes	☐ Yes	☒ No	Service Animal	☐ Yes	☒ No
Insulin	☐ Yes	☐ No			
Diabetes related Oral Meds	☐ Yes	☐ No			

Explain (O2 / infection / device / hospice): Pneumonia, improving on oral antibiotics. Off oxygen since 08/26

BEHAVIORAL AND MENTAL HEALTH STATUS

Does the patient have any limitations, behavioral or mental health challenges? ☐ Yes ☒ No

☐ Mental Health DX If checked, what type?

☐ Non-compliant ☐ Forgetful ☐ Cognitive Impairment ☐ Registered Sex Offender

☐ Other Explain:

Length of Stay Authorized in Recuperative Care: 30 Days

ILLUMINATION STAFF ONLY

Date Received: _____ Reviewed by: _____

Approved: [ ] Yes [ ] No Date: _____ Authorization #: _____ Reason for Denial: _____

Admission Date: _____ Admission Time: _____ Exit Date: _____

SYNTHETIC RECORD FOR A HACKATHON - NOT A REAL PATIENT - Claude Impact Lab OC 2026

Patient Face Sheet

Printed 08/27/2026 11:05 by Rachel Imani

PATIENT INFORMATION

Patient	BENNING, HAROLD	**Sex**	M
DOB	01/30/1960	**Age**	66y
Address	NO FIXED ADDRESS	**Language**	English
Phone	NONE	**Marital Status**	Single
Emergency Contact	None listed	**Interpreter**	No

INSURANCE INFORMATION

Primary	Medi-Cal managed care plan	**Member ID**	SYN-M-296481
Secondary	None	**Financial Class**	Medi-Cal Managed Care

ENCOUNTER INFORMATION

Admit Date/Time	08/24/2026 06:32	**Mode of Arrival**	Ambulance
Patient Class	Inpatient	**Unit**	5 Tower Pulmonary
Admitting Diagnosis	Community-acquired pneumonia, right lower lobe, with COPD exacerbation	**Diagnosis Code**	J18.9, J44.0, J44.1, Z59.02
Attending	Samuel Oyelaran, MD	**Expected Discharge**	08/28/2026

Bayfront University Medical Center, 1 University Health Plaza, Orange County, CA. Main 714-555-0130.

BENNING, HAROLD (MRN: SYN-407192) DOB: 01/30/1960 66y M

Bayfront University Medical Center · 5 Tower Pulmonary

Authorization Request: Recuperative Care (Community Support)

Submitted 08/27/2026 11:05 by Rachel Imani

Health Plan	Medi-Cal managed care plan
Member ID	SYN-M-296481
Service Requested	Recuperative care (medical respite)
Requested Provider	Illumination Health + Home
Days Requested	30
Diagnosis Code(s)	J18.9, J44.0, J44.1, Z59.02
Status	Pending health plan review
Authorization #	Not yet assigned

CLINICAL JUSTIFICATION

Recuperative care referral. 66-year-old man with pneumonia and COPD. Off oxygen since 08/26, on oral antibiotics, walking on his own. No home oxygen needed. Unhoused. Ready 08/28.

BENNING, HAROLD (MRN: SYN-407192) DOB: 01/30/1960 66y M Bayfront University Medical Center · 5 Tower Pulmonary

History and Physical

Samuel Oyelaran, MD - Admission date 08/24/2026 - Signed

CHIEF COMPLAINT

Cough, fever and shortness of breath for 4 days.

HISTORY OF PRESENT ILLNESS

66-year-old man with COPD who presents with 4 days of productive cough, fever and worsening shortness of breath. Temperature 101.8 F, oxygen saturation 88% on room air on arrival, improving to 94% on 2 liters by nasal cannula. Chest X-ray shows a right lower lobe infiltrate.

Started ceftriaxone and azithromycin, nebulized bronchodilators and prednisone. Admitted to the pulmonary unit. He does not use oxygen at home.

PAST MEDICAL HISTORY

- COPD, moderate. FEV1 58% of predicted in 2025
- Hypertension
- Former smoker, quit 2023
- Gastroesophageal reflux

PAST SURGICAL HISTORY

None.

HOME MEDICATIONS

- Tiotropium 18 mcg inhaled daily
- Albuterol inhaler, 2 puffs every 4 to 6 hours as needed
- Amlodipine 5 mg by mouth daily

ALLERGIES

No known drug allergies.

SOCIAL HISTORY

Lost his apartment 5 months ago. Stays outdoors or in an emergency shelter when a bed is open. No alcohol or drug use. Manages his own inhalers and pills.

PHYSICAL EXAM

General: Awake, alert, mildly short of breath with talking.

HEENT: Normocephalic. Mucous membranes moist.

Cardiovascular: Regular rate and rhythm. No murmurs.

Respiratory: Crackles at the right base. Scattered expiratory wheezes. Speaking full sentences.

Abdomen: Soft, non-tender, non-distended.

Extremities: No edema.

Skin: Warm and dry.

Neuro: Alert and oriented x4. No focal deficits.

Psych: Calm, cooperative. Normal mood and affect.

ASSESSMENT AND PLAN

- Right lower lobe pneumonia: ceftriaxone and azithromycin, change to oral when improving.
- COPD exacerbation: nebulizers, prednisone 40 mg for 5 days, wean oxygen as tolerated.
- Hypertension: continue amlodipine.
- Disposition: unhoused. Case management consult.

BENNING, HAROLD (MRN: SYN-407192) DOB: 01/30/1960 66y M

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Diagnostic Imaging Report

Chest X-ray, 2 views - 08/24/2026 06:55

Indication: Cough, fever, low oxygen.

FINDINGS

Airspace consolidation in the right lower lobe. Hyperinflated lungs. No pleural effusion. No pneumothorax. Heart size normal.

IMPRESSION

Right lower lobe pneumonia. Changes of COPD.

Electronically signed by D. Okafor, MD

SYNTHETIC RECORD FOR A HACKATHON - NOT A REAL PATIENT - Claude Impact Lab OC 2026

BENNING, HAROLD (MRN: SYN-407192) DOB: 01/30/1960 66y M Bayfront University Medical Center · 5 Tower Pulmonary

Diagnostic Imaging Report

Chest X-ray, 1 view - 08/27/2026 07:20

Indication: Follow-up pneumonia.

FINDINGS

Right lower lobe consolidation is smaller and less dense than on 08/24. No effusion. No pneumothorax.

IMPRESSION

Improving right lower lobe pneumonia.

Electronically signed by D. Okafor, MD

BENNING, HAROLD (MRN: SYN-407192) DOB: 01/30/1960 66y M

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Laboratory Results

Collected 08/27/2026 05:15

COMPLETE BLOOD COUNT

Test	Result	Reference	Units
WBC	9.1	4.0-11.0	K/uL
Hemoglobin	14.5	12.0-17.0	g/dL
Hematocrit	43.5	36-50	%
Platelets	287	150-400	K/uL

BASIC METABOLIC PANEL

Test	Result	Reference	Units
Sodium	139	135-145	mmol/L
Potassium	4.2	3.5-5.1	mmol/L
Chloride	103	98-107	mmol/L
CO2	25	22-29	mmol/L
BUN	12	7-20	mg/dL
Creatinine	1.0	0.6-1.3	mg/dL
Glucose	88	70-99	mg/dL

TRENDS AND OTHER RESULTS

Test	Collected	Result	Reference
WBC	08/24, 08/25, 08/27	14.6, 12.0, 9.1 K/uL	4.0-11.0
Procalcitonin	08/24/2026 07:05	1.8 ng/mL	<0.25

MICROBIOLOGY

Test	Collected	Result
Blood culture x2	08/24/2026 07:10	No growth at 72 hours
Sputum culture	08/24/2026 08:30	Normal respiratory flora
MRSA nares PCR	08/24/2026 09:00	NOT DETECTED
SARS-CoV-2, influenza, RSV PCR	08/24/2026 07:00	ALL NOT DETECTED
SARS-CoV-2 antigen	08/27/2026 10:15	NEGATIVE

BENNING, HAROLD (MRN: SYN-407192) DOB: 01/30/1960 66y M Bayfront University Medical Center · 5 Tower Pulmonary

Progress Notes

08/26/2026 09:30 Samuel Oyelaran, MD Hospital Medicine Progress Note

Breathing easier. Off supplemental oxygen since 07:00 today. Saturation 94 to 96% at rest on room air. Afebrile for 24 hours. Changed to oral amoxicillin-clavulanate. Completed 3 days of azithromycin.

08/27/2026 09:10 Samuel Oyelaran, MD Hospital Medicine Progress Note

Afebrile for 48 hours. Room air saturations 94 to 96% at rest and 93 to 94% walking. He does not need and does not qualify for home oxygen. Using his inhalers correctly.

Ready for discharge 08/28 to finish antibiotics through 08/31. Last prednisone dose 08/28.

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BENNING, HAROLD (MRN: SYN-407192) DOB: 01/30/1960 66y M

Bayfront University Medical Center · 5 Tower Pulmonary

Respiratory Therapy Consult

Sofia Paredes, RRT - 08/27/2026 10:40 - Signed

Reason for consult: Oxygen assessment and inhaler teaching before discharge.

Breathing treatments this admission: albuterol-ipratropium by nebulizer every 6 hours through 08/26, then changed to metered-dose inhaler with spacer.

Walking oximetry on room air: 95% at rest, 93% after walking 200 ft, back to 95% within one minute. No oxygen needed at rest or with activity. Patient showed correct inhaler and spacer technique on teach-back.

RECOMMENDATIONS

- No home oxygen.
- Continue tiotropium daily and albuterol as needed with spacer.

BENNING, HAROLD (MRN: SYN-407192) DOB: 01/30/1960 66y M

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Vital Signs Flowsheet

Date/Time	BP	HR	RR	Temp F	SpO2	O2
08/26/2026 20:00	132/80	84	18	98.6	95%	Room air
08/27/2026 00:00	128/78	80	18	98.2	94%	Room air
08/27/2026 04:00	126/76	78	16	98.0	95%	Room air
08/27/2026 08:00	130/80	82	18	98.3	96%	Room air
08/27/2026 10:30	134/82	86	18	98.4	95%	Room air

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BENNING, HAROLD (MRN: SYN-407192) DOB: 01/30/1960 66y M Bayfront University Medical Center · 5 Tower Pulmonary

Physical Therapy Evaluation

Priscilla Nwosu, PT, DPT - 08/26/2026 15:00 - Signed

Area	Finding
Bed mobility	Independent
Transfers	Independent
Ambulation	250 ft, independent, no device, one standing rest break
Balance	Good
Assistive device	None

ASSESSMENT

Mild shortness of breath with exertion that settles with a short rest. Safe and independent.

Discharge recommendation: No PT follow-up needed.

BENNING, HAROLD (MRN: SYN-407192) DOB: 01/30/1960 66y M

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Nursing Assessment / ADL Flowsheet

L. Hartigan, RN - 08/27/2026 07:15

Item	Assessment
Orientation	Alert and oriented x4
Bathing	Independent
Dressing	Independent
Toileting	Independent. Continent of bowel and bladder
Feeding	Independent
Medications	Uses own inhalers and takes own pills with RN observing
Fall risk	Low
Skin	Intact
Isolation	None
Safety screen	Suicide risk screen negative. Denies thoughts of self-harm

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Discharge Medication List

Reconciled 08/27/2026

Medication	Dose	Route	Frequency	Notes
Amoxicillin-clavulanate	875/125 mg	By mouth	Twice daily	Through 08/31/2026
Prednisone	40 mg	By mouth	Daily	Last dose 08/28/2026
Tiotropium	18 mcg	Inhaled	Daily	Home medication
Albuterol inhaler with spacer	2 puffs	Inhaled	Every 4 to 6 hours as needed	Home medication
Amlodipine	5 mg	By mouth	Daily	Home medication

30-day supply to bedside before discharge.

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Case Management / Social Work Note

Rachel Imani, RN Case Manager - 08/27/2026 10:55

Patient lost his apartment 5 months ago and has been outdoors or in emergency shelter since. He needs a place to rest and finish antibiotics while his breathing recovers. Agreeable to recuperative care.

DISCHARGE PLANNING

Item	Status
Housing	Unhoused 5 months.
Oxygen	None needed. No equipment ordered.
Follow-up	Primary care 09/03/2026 14:00. Pulmonary clinic 09/17/2026.
Disposition	Recuperative care referral faxed 08/27/2026.
Health plan authorization	Submitted 08/27/2026. Pending.

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